

A Career-Defining Opportunity in U.S. Dentistry

For the Dentist Partner · Confidential · October 2026

You spent years mastering your craft. You chose dentistry to care for patients — not to manage payroll, negotiate leases, deal with insurance billing, or spend evenings on paperwork. Yet that's what owning a solo practice often becomes.

This proposal is built around a different idea: **what if you could practice dentistry in the United States, focus entirely on clinical work, and at the same time build real equity in a growing business?**

The short version: We are building a network of dental clinics in the U.S., structured as a Dental Support Organization (DSO). We are looking for a dentist partner who brings clinical excellence, a connection to the Hispanic community, and the ambition to own a stake in something much larger than a single practice. The operational infrastructure — management, marketing, purchasing, HR — is handled by the promoter team. You focus on patients.

1. Solo Practice vs. DSO Partner — The Real Comparison

Most dentists who open their own practice in North America face the same reality within a few years:

SOLO PRACTICE

- \$400K–\$600K startup cost, mostly debt
- You are the dentist AND the manager
- Revenue tied entirely to your chair time
- No scalable equity — practice value dies with you
- Administrative burden grows as the practice grows

DSO PARTNER

- Competitive salary from day one
- You focus entirely on clinical work
- Equity stake in a multi-clinic network
- Scalable value — grows as the network grows
- Operations, admin and marketing handled centrally

The DSO model is not new. It is how **30% of U.S. dentistry** already operates, and that share is growing every year. The question is whether you participate as an owner or as an employee.

2. Why the U.S. — And Why Now

The U.S. dental market exceeds **\$180 billion annually** and grows at 5.5% per year. But the most relevant number for this opportunity is this:

65+ million Hispanic residents in the U.S. — and ~40% lack regular access to dental care. This is the fastest-growing population segment in the country, concentrated in Florida, Texas, California, and New York. They are underserved not for lack of willingness to pay, but for lack of providers who speak their language and understand their culture. A bilingual clinic with Latin-rooted dentists has an immediate competitive advantage that no amount of marketing can replicate.

DSO valuations are also at historically high multiples — 6x to 12x EBITDA. Groups like Aspen Dental, Pacific Dental, and Heartland Dental are actively acquiring smaller networks. **Building now, with the intention to sell in 4–6 years, captures that window.**

3. Your Role as a Partner Dentist

Clinically

- Lead the clinical team at your assigned clinic
- Set treatment standards and mentor associate dentists
- Serve a patient base that values bilingual, culturally aligned care
- Full clinical autonomy — treatment decisions are yours

As a Co-Investor

- Equity stake of **5%–15% per clinic**, acquired through capital contribution or clinical commitment
- Profit participation on top of salary
- Option to increase your stake as the network expands
- Full participation in the exit event — the network sale

What You Don't Have to Do

- Manage the lease, build-out or equipment procurement
- Handle payroll, HR or staff disputes
- Run marketing or manage social media
- Deal with insurance billing and collections
- Negotiate with suppliers

All of that is handled by the promoter team. Your time is for patients.

4. What This Looks Like Financially

Annual Income (Stabilized Clinic, Year 2–3)

Income Stream	Estimated Range
Base salary as clinical lead	\$180,000 – \$250,000 / year
Profit participation (your % of clinic EBITDA)	\$25,000 – \$100,000 / year
Total annual compensation	\$200,000 – \$350,000 / year

Exit Value — The Big Picture

This is where the DSO model creates generational wealth. When the network is sold to a larger group or private equity fund:

Network at Sale	Estimated Valuation	Your 10% Stake
3 clinics, Year 3–4	\$8M – \$15M	\$800K – \$1.5M
5 clinics, Year 5	\$16M – \$32M	\$1.6M – \$3.2M
8+ clinics, Year 6–7	\$30M – \$60M	\$3M – \$6M

A solo practice in the U.S. typically sells for 60%–80% of one year's collections — roughly \$900K–\$1.5M after decades of work and personal financial risk. A 10% stake in a 5-clinic DSO network at exit can match or exceed that in a fraction of the time, without the debt and without the administrative burden.

5. Your Path from Canada to U.S. Practice

As a dentist licensed and practicing in Canada, your pathway to U.S. licensure is the most direct available to any foreign-trained dentist:

Canadian dental programs accredited by CDAC are fully recognized by CODA (the U.S. accrediting body), by reciprocal agreement confirmed by the American Dental Association. This means you do not need to repeat dental school or complete an Advanced Standing Program.

The Process

- **INBDE** (Integrated National Board Dental Examination) — written exam, taken independently, preparation typically 6–12 months
- **Clinical examination** — state-specific or ADEX, evaluates hands-on skills
- **State license application** — paperwork, fees, background check, credential verification

Estimated total timeline: 12–18 months from decision to licensed. The promoter team provides administrative support throughout this process.

Compare this to a Venezuelan or Colombian-trained dentist, who in most U.S. states must complete a 2-year Advanced Standing Program before even taking the board exams — a process that costs \$80K–\$150K and takes 3–5 years. Your Canadian credentials put you years ahead.

6. How the Ownership Works

The legal structure follows the standard U.S. DSO model:

- **DSO / LLC (holding company)** — handles operations, management, marketing. This is where the investment and economic ownership sit. You participate here as a co-investor.
- **Professional Corporation (PC/PLLC)** — holds the dental license and employs the clinical staff, as required by U.S. law. A U.S.-licensed dentist must be a nominal owner; this is being arranged as part of Phase 1.

Your equity, your salary, and your exit proceeds all flow through the DSO structure. This is how every major dental group in the U.S. is organized.

7. Is This for You?

This opportunity is designed for a dentist who:

- Wants to practice in the U.S. market and serve the Hispanic community
- Values clinical autonomy without the burden of solo ownership
- Is willing to invest capital or clinical commitment in exchange for equity
- Is ready to begin or has already started the U.S. licensing process
- Sees the exit event — not just annual income — as part of the long-term plan

No capital commitment is required at this stage. The next step is an exploratory conversation to align on vision, discuss your current licensing status, and understand how you might fit into the structure — as a clinical lead, a co-investor, or both.

Proposed Next Steps

Step 1 — Exploratory Call (this week)

Informal conversation to validate mutual interest and discuss your current situation and timeline.

Step 2 — Due Diligence (Month 1)

Review licensing status, discuss equity terms, identify target markets together.

Step 3 — Letter of Intent (Month 2)

Non-binding document formalizing mutual interest and role before any legal expenses.

Step 4 — Legal Formation & First Round Close (Months 3–6)

Form the LLC with a U.S. dental attorney and close the first capital commitments.

This document was prepared as a starting point for conversation. Financial projections are directional estimates based on public U.S. DSO sector benchmarks. Compensation and equity figures are illustrative and subject to negotiation. Specialized legal and financial counsel is strongly recommended before any formal commitment.